RESULTS

4.0 Introduction

This chapter critically analyses the findings presented in the tables and charts concerning factors influencing self-medication and its impact on the utilization of health facilities among students at Ondo State College of Health Technology, Akure.

The data is discussed in depth, with a breakdown of the socio-demographic characteristics, prevalence of self-medication, students' perceptions, attitudes, and key influencing factors, as well as the impact on healthcare-seeking behaviour. Each table and figure is critically analysed to provide a thorough understanding of the results.

The results are organized by each table and chart for detailed clarity in the following subheadings:

4.1 Socio-demographic Characteristics of students at Ondo State College of Health Technology, Akure

4.2 The prevalence of self-medication among students of Ondo State College of Health Technology, Akure

4.3 The Perception of the Students to Self-medication in Ondo State College of Health Technology, Akure

4.4 The Attitude of Students towards Self-Medication in Ondo State College of Health Technology, Akure

4.5 The Key Factors Influencing Self-Medication Practices among Students of Ondo State College of Health Technology, Akure

4.6 The Impact of Self-Medication on the Utilization of Health Facility by the Students of Ondo State College of Health Technology, Akure

4.7 Binary Logistic Regression Model showing predictors of health facility utilization by the students of Ondo State College of Health Technology, Akure (n = 380)

4.1 Socio-demographic Characteristics of students at Ondo State College of Health Technology, Akure

Table 4:1. Distribution of Socio-demographic Characteristics of the Respondents

Variables	Frequency (n)	Percentage (%)
Gender:		
Male	127	33.4
Female	253	66.6
Age of respondents		
15 – 19 years	23	6.1
20 – 24 years	332	87.4
25 – 29 years	19	5.0
30 – 34 years	2	0.5
35 – 39 years	4	1.1
Department:		
Community Health	95	25.0
Environmental Health	61	16.1
Pharmacy Technician	33	8.7
Medical Laboratory Technician	58	15.3
Health Information Management	28	7.4
Dental Surgery Technician	61	16.1
Health Care Giving	41	10.8
Public Health Nursing	3	0.8
Level of Study:		
100 Level	180	47.4
200 Level	115	30.3
300 Level	85	32.4
Marital Status:		
Single	366	96.3
Married	14	3.7
Total	**380**	**100.0**

Field work, 2024

Table 4.1 summarizes the socio-demographic characteristics of the respondents, highlighting variations across gender, age, department, level of study, and marital status. Of the 380 respondents, a significant majority (66.6%) are female, while males make up only 33.4%. This gender distribution suggests that female students may have a higher presence in this institution, which could influence general health practices observed among the population.

The age distribution shows that the majority of respondents (87.4%) fall within the 20-24 years age range, indicative of a young adult demographic. Students aged 15-19 years constitute only 6.1%, while older age brackets (25-29 years, 30-34 years, and 35-39 years) make up a smaller proportion, collectively amounting to 6.6%. This concentration of younger adults may reflect particular health-seeking behaviours and attitudes toward self-medication often seen in this age group, including a potential tendency toward convenience and independence in health management.

Departmental distribution reveals that Community Health students are the largest group (25%), followed by Environmental Health and Dental Surgery Technician, each representing 16.1%. Public Health Nursing students, at 0.8%, are the smallest group. This departmental variance may impact overall familiarity with and attitudes toward healthcare practices, with those in health-related disciplines potentially having more awareness of medical information and resources.

Regarding academic levels, 100-level students constitute the largest proportion at 47.4%, followed by 200-level students at 30.3%, and 300-level students at 32.4%. This range in study levels can influence students' awareness and knowledge of health practices, with more advanced students potentially having greater exposure to health education. Finally, the majority of students (96.3%) are single, with only 3.7% married, suggesting that familial responsibilities, which can affect healthcare decision-making, are not a major factor within this student population.

4.2 The prevalence of self-medication among students of Ondo State College of Health Technology, Akure

Table 4.2: Prevalence of Self-Medication in Ondo State College of Health Technology, Akure (n = 380)

	Yes n (%)	No n (%)
Ever utilized the school's health facilities in the past six months	290 (76.3)	90 (23.7)
Engaged in self-medication in the past six months	347 (91.3)	33 (8.7)
Indications for self-medication		
Headache	305 (87.9)	42 (12.1)
Cough and Cold	288 (83.0)	59 (17.0)
Fever	291 (83.9)	56 (16.1)
Allergies (body reaction to substances)	276 (79.5)	71 (20.5)
Pain (e.g., muscle pain, joint pain)	296 (85.3)	51 (14.7)

Field work, 2024

Table 4.2 examines the prevalence and primary indications for self-medication among the students. The findings are striking, as 91.3% of respondents reported engaging in self-medication over the past six months, a clear indication that self-medication is a common practice among students. This high prevalence suggests that students frequently choose self-care over visiting health facilities for various reasons, such as convenience or financial constraints.

Regarding health facility utilization, 76.3% of students visited the institution's health facility within the past six months, while 23.7% did not. The high utilization rate of school health facilities may indicate an awareness of available services; however, the simultaneous high self-medication rate suggests that students often prefer self-treatment for certain conditions.

The most common reasons for self-medication include headaches (87.9%), pain such as muscle or joint pain (85.3%), fever (83.9%), cough and cold (83.0%), and allergic reactions (79.5%). This trend shows that students primarily self-medicate for common,

manageable symptoms, which may not be seen as serious enough to warrant professional medical intervention. Headaches, pain, and fevers—being the top three indications—may reflect stress or lifestyle factors associated with academic demands.

4.3 The Perception of the Students to Self-medication in Ondo State College of Health Technology, Akure

Table 4.3: Students' perception to self-medication in Ondo State College of Health Technology, Akure (n = 380)

	Yes n (%)	No n (%)
I believe that self-medication is a cost-effective way to manage common health issues while studying	343 (90.3)	37 (9.7)
I perceive self-medication as a means to quickly address health problems without delays in seeking professional care.	354 (93.2)	26 (6.8)
I think self-medication practices among students in my institution are common and widely accepted.	359 (94.5)	21 (5.5)
I am aware of the potential dangers associated with self-medication, such as incorrect dosages and masking underlying health conditions	370 (97.4)	10 (2.6)
My institution should provide more education on responsible self-medication to ensure student safety	259 (68.2)	121 (31.8)

Field work, 2024

Table 4.3 and Figure 4.1 reveal the students' perceptions of self-medication, providing insights into their attitudes and beliefs about its effectiveness and potential risks. An overwhelming 90.3% of respondents consider self-medication to be a cost-effective way to manage common health issues, illustrating that financial considerations play a significant role in students' health practices. Additionally, 93.2% of students perceive self-medication as a quick method to address health issues without delay, underscoring the appeal of accessibility and immediacy in managing health concerns.

A substantial 94.5% believe that self-medication is a widely accepted practice among their peers, suggesting a normalization of self-medication within the student community. Furthermore, 97.4% are aware of the risks associated with self-medication, such as incorrect dosages or masking underlying health issues, which reflects a critical awareness among students about the potential downsides of self-care practices. However, despite recognizing these risks, only 68.2% believe that the

institution should provide more education on responsible self-medication practices, which could indicate a gap in health promotion or a potential underestimation of the dangers involved.

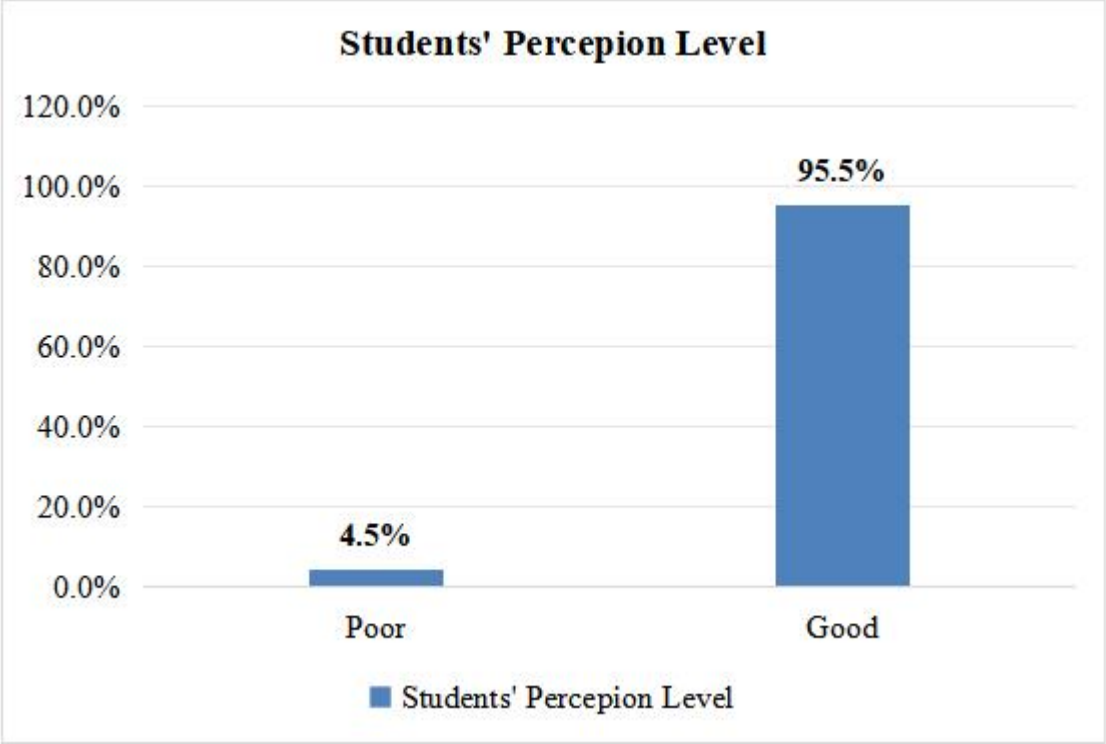

Figure 4.1: Level of Students' Perception to Self-medication

Figure 4.1 provides a graphical representation of the levels of perception among students regarding self-medication. The figure categorizes perception into levels based on responses to statements about the perceived cost-effectiveness, convenience, acceptance, and awareness of risks associated with self-medication.

Overall, Figure 4.1 illustrates a high level of acceptance and positive perception towards self-medication among students, driven by financial and practical considerations, though tempered by an awareness of possible health risks.

4.4 The Attitude of Students towards Self-Medication in Ondo State College of Health Technology, Akure

Table 4.4: Students' attitude towards self-medication in Ondo State College of Health Technology, Akure (n = 380)

	Yes n (%)	No n (%)
I believe that self-medication is a convenient way to address minor health issues without consulting a healthcare professional	332 (87.4)	48 (12.6)
I am confident in my ability to make informed decisions about self-medication, including choosing the right over-the-counter (OTC) medications	330 (86.8)	50 (13.2)
I think self-medication helps reduce the burden on healthcare facilities, making it easier for people with severe health issues to access care	242 (63.7)	138 (36.3)
I am aware of the potential risks associated with self-medication, such as adverse drug reactions and misdiagnosis	348 (91.6)	32 (8.4)
I believe that self-medication should only be used for non-serious health problems, and individuals should seek professional medical advice for more severe conditions	332 (87.4)	48 (12.6)

Field work, 2024

Table 4.4 and Figure 4.2 delve into the attitudes students hold regarding self-medication. A significant 87.4% view it as a convenient solution for minor health issues without consulting healthcare professionals, while 86.8% feel confident in making informed choices about over-the-counter medications. This confidence in self-management points to a high level of perceived autonomy in handling personal health.

Moreover, 63.7% believe that self-medication alleviates the burden on healthcare facilities, allowing those with severe health issues better access to medical resources. This perception may reflect an understanding of healthcare systems' limitations and an altruistic approach to healthcare utilization. Notably, 91.6% express awareness of the risks, such as adverse reactions or misdiagnosis, demonstrating a balanced attitude toward self-medication as a useful, albeit potentially risky, practice. Importantly,

87.4% agree that self-medication should be limited to non-serious conditions, suggesting a conscientious approach in reserving professional advice for more serious health concerns.

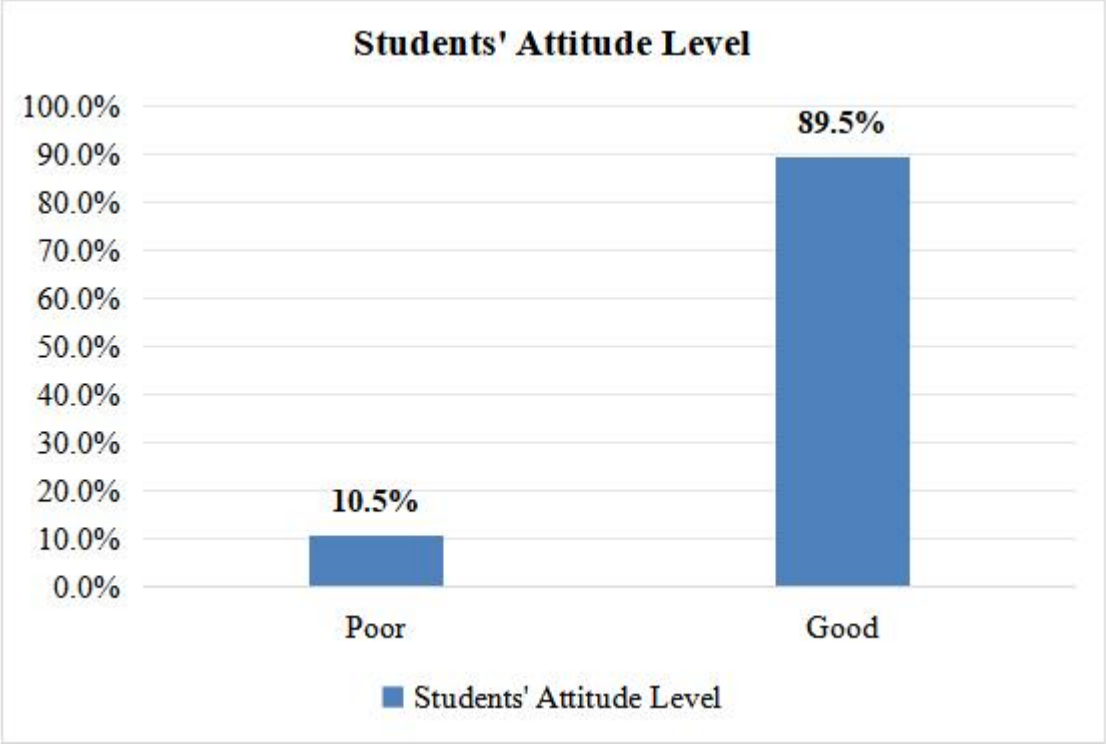

Figure 4.2: Level of Students' Attitude towards Self-medication

Figure 4.2 presents data on the level of students' attitudes towards self-medication, showing how attitudes align with actual self-medication practices and preferences. Students' responses reflect a generally favourable attitude towards managing minor health issues independently, suggesting self-confidence in addressing certain health concerns without healthcare provider involvement.

Figure 4.2 thus illustrates a strong, predominantly positive attitude towards self-medication, characterized by a high degree of confidence and a measured understanding of its limitations. This attitude aligns closely with the observed high prevalence of self-medication practices, emphasizing students' readiness to independently manage certain health needs while being cognizant of potential risks.

4.5 The Key Factors Influencing Self-Medication Practices among Students of Ondo State College of Health Technology, Akure

Table 4.5: Key factors of self-medication practices in Ondo State College of Health Technology, Akure (n = 380)

	Yes n (%)	No n (%)
Lack of access to healthcare facilities or professionals	30 (7.9)	350 (92.1)
Previous successful experiences with self-medication	302 (79.5)	78 (20.5)
Influence of friends who self-medicate	292 (76.8)	88 (23.2)
Influence of family members who self-medicate	294 (77.4)	86 (22.6)
Limited knowledge about the risks of self-medication	296 (77.9)	84 (22.1)
Perceived inconvenience of seeking professional medical advice	43 (11.3)	337 (88.7)
Easy availability of over-the-counter medications	310 (81.6)	70 (18.4)
Financial constraints affecting access to healthcare services	288 (75.8)	92 (24.2)

Field work, 2024

Table 4.5 presents the primary factors influencing self-medication practices among students. The findings reveal that previous successful experiences with self-medication are a leading factor, with 79.5% of students acknowledging this as a motivation. This trend suggests that familiarity with self-treatment and positive outcomes in the past reinforce the likelihood of future self-medication behaviors.

The influence of friends and family also plays a substantial role, with 76.8% and 77.4% of respondents, respectively, stating that they are influenced by these groups. This strong social influence highlights the importance of peer and family norms in shaping health behaviors, indicating that students may adopt self-medication practices based on the examples set by close contacts.

Limited knowledge about the risks of self-medication is cited by 77.9% of respondents as a contributing factor, which points to an information gap that might be addressed through educational interventions. Financial constraints affect 75.8% of

students' decisions, underscoring the role of economic factors in health choices, as limited financial resources restrict access to professional healthcare.

Lastly, the ease of access to over-the-counter medications is acknowledged by 81.6% of students as a facilitating factor. The availability of medications without prescription likely encourages self-medication, as students can readily obtain drugs for self-care without the need for formal consultations.

Table 4.6: Key factors influencing self-medication practices in Ondo State College of Health Technology, Akure (n = 380)

Variables	Self-medication practices Yes N (%)	Self-medication practices No n (%)	Number of Students	P-value
Lack of access to healthcare facilities or professionals:				
Yes	22 (6.3)	8 (24.2)	30	**0.002**
No	325 (93.7)	25 (75.8)	350	
Previous successful experiences with self-medication:				
Yes	295 (85.0)	7 (21.2)	302	**<0.0001**
No	52 (15.0)	26 (78.8)	78	
Friends who self-medicate:				
Yes	284 (81.8)	8 (24.2)	292	**<0.0001**
No	63 (18.2)	25 (75.8)	88	
Family members who self-medicate:				
Yes	285 (82.1)	9 (27.3)	294	**<0.0001**
No	62 (17.9)	24 (72.7)	86	
Limited knowledge about the risks of self-medication:				
Yes	289 (83.3)	7 (21.2)	296	**<0.0001**
No	58 (16.7)	26 (78.8)	84	
Perceived inconvenience of seeking professional medical advice:				
Yes	34 (9.8)	9 (27.3)	43	**<0.0001**
No	313 (90.2)	24 (72.7)	337	
Easy availability of over-the-counter medications:				
Yes	301 (86.7)	9 (27.3)	310	**<0.0001**
Nol	46 (13.3)	24 (72.7)	70	
Financial constraints affecting access to healthcare services:				
Yes	280 (80.7)	8 (24.2)	288	**<0.0001**
No	67 (19.3)	25 (75.8)	92	
Total	**347 (100.0)**	**33 (100.0)**	**380**	

Table 4.6 explores the statistical significance of key factors in relation to self-medication practices. The p-values indicate strong correlations for most factors:

Previous successful experiences have a significant impact ($p < 0.0001$), with 85.0% of those with positive self-medication experiences continuing to self-medicate, compared to only 21.2% of those without such experiences.

Social influence from friends and family is also significant ($p < 0.0001$), with 81.8% of students influenced by friends who self-medicate, in contrast to 24.2% among those without such influences. Similarly, 82.1% of students with family influences practice self-medication.

Knowledge limitations ($p < 0.0001$) and **ease of access to medications** ($p < 0.0001$) further reinforce self-medication, as students lacking full awareness of risks or with easy access to medications are more likely to self-medicate.

This analysis points to both experiential and environmental factors as strongly predictive of self-medication, with previous experiences, social influences, and access playing crucial roles.

Table 4.7: Socio-demographic factors influencing self-medication practices in Ondo State College of Health Technology, Akure (n = 380)

Variables	Self-medication practices Yes n (%)	Self-medication practices No n (%)	Number of Students	P-value
Gender:				
Male	117 (33.7)	10 (30.3)	127	0.847
Female	230 (66.3)	23 (69.7)	253	
Age of respondents				
15 – 19 years	19 (5.5)	4 (12.1)	23	**0.012**
20 – 24 years	309 (89.0)	23 (69.7)	332	
25 – 29 years	15 (4.3)	4 (12.1)	19	
30 – 34 years	1 (0.3)	1 (3.0)	2	
35 – 39 years	3 (0.9)	1 (3.0)	4	
Department:				
Community Health	88 (25.4)	7 (21.2)	95	**<0.0001**
Environmental Health	61 (17.6)	0 (0.0)	61	
Pharmacy Technician	32 (9.2)	1 (3.0)	33	
Medical Laboratory Technician	54 (15.6)	4 (12.1)	58	
Health Information Management	18 (5.2)	10 (30.3)	28	
Dental Surgery Technician	51 (14.7)	10 (30.3)	61	
Health Care Giving	40 (11.5)	1 (3.0)	41	
Public Health Nursing	3 (0.9)	0 (0.0)	3	
Level of Study:				
100 Level	179 (51.6)	1 (3.0)	180	**<0.0001**
200 Level	103 (29.7)	12 (36.4)	115	
300 Level	65 (18.7)	20 (60.0)	85	
Marital Status:				
Single	333 (96.0)	33 (100.0)	366	0.621
Married	14 (4.0)	0 (0.0)	14	
Total	**347 (100.0)**	**33 (100.0)**	**380**	

Table 4.7 examines how socio-demographic characteristics correlate with self-medication practices. Gender shows a minor effect, with 33.7% of males and 66.3% of females engaging in self-medication (p = 0.847), suggesting no significant gender difference.

Age, however, reveals a more noteworthy pattern (p = 0.012). The highest self-medication rates are among students aged 20-24 years, with 89.0% of self-medicators

in this group, while other age groups, including 15-19 years and 25-29 years, show much lower frequencies.

Department and level of study also significantly impact self-medication rates (both $p < 0.0001$). Students in Community Health (25.4%) and Environmental Health (17.6%) departments report the highest self-medication rates. Furthermore, first-year students (100 Level) show higher self-medication frequencies (51.6%) than students in upper levels, which may reflect lower awareness or access to healthcare resources among newer students.

Table 4.8: Additional factors influencing self-medication practices in Ondo State College of Health Technology, Akure (n = 380)

Variables	Self-medication practices		Number of Students	P-value
	Yes n (%)	No n (%)		
Perception level to self-medication:				
Poor	8 (2.3)	9 (27.3)	17	**<0.0001**
Good	339 (97.7)	24 (72.7)	363	
Attitude level towards self-medication:				
Poor	24 (6.9)	16 (48.5)	366	**<0.0001**
Good	323 (93.1)	17 (51.5)	14	
Total	**347 (100.0)**	**33 (100.0)**	**380**	

Table 4.8 considers other influential factors, specifically students' perceptions and attitudes. Perception level is strongly associated with self-medication ($p < 0.0001$); students with a "good" perception of self-medication are more likely to engage in the practice (97.7%) than those with a "poor" perception (2.3%). Similarly, attitude levels show a significant correlation ($p < 0.0001$), with students holding a positive attitude towards self-medication engaging more frequently (93.1%).

These findings indicate that personal beliefs about the effectiveness and convenience of self-medication are strong motivators, likely reinforcing behaviours that are perceived as beneficial or low-risk.

4.6 The Impact of Self-Medication on the Utilization of Health Facility by the Students of Ondo State College of Health Technology, Akure

Table 4.9: Impact of self-medication on the utilization of health facility by the students of Ondo State College of Health Technology, Akure (n = 380)

	Yes n (%)	No n (%)
Has self-medication influenced your decision to delay visiting a healthcare facility for a more serious health concern?	310 (81.6)	70 (18.4)
Do you feel that self-medication reduces the frequency of your visits to health facilities for minor ailments?	347 (91.3)	33 (8.7)
Have you ever chosen self-medication over seeking professional medical advice because of the cost of healthcare services?	347 (91.3)	33 (8.7)
Has self-medication led to any complications that required subsequent visits to a healthcare facility?	47 (12.4)	333 (87.6)
Do you believe that frequent self-medication reduces your trust in the need for professional healthcare services for minor health issues?	347 (91.3)	33 (8.7)
Do you think self-medication has a long-term impact on students' reliance on healthcare facilities for proper medical attention?	305 (80.3)	75 (19.7)

Field work, 2024

Table 4.9 investigates the impact of self-medication on students' decisions to use healthcare facilities. A large majority (81.6%) report that self-medication has influenced their decision to delay visits to a healthcare facility for serious health concerns. This suggests that self-medication may be leading students to postpone necessary medical care, potentially worsening health outcomes over time.

Moreover, 91.3% believe that self-medication reduces the frequency of their healthcare visits for minor issues, indicating that they view self-medication as an effective way to manage common ailments independently. Cost is a significant factor, as 91.3% report opting for self-medication over seeking professional care due to financial concerns, highlighting the economic drivers behind health choices.

This section indicates that while self-medication offers an immediate, cost-effective solution for minor health issues, it may lead to delayed professional care, particularly for more serious conditions.

Table 4.10: Impact of self-medication factors on utilization of health facility by the students of Ondo State College of Health Technology, Akure (n = 380)

Variables	Utilization of health facility		Number of Students	P-value
	Yes N (%)	No n (%)		
Self-medication caused decision to delay visiting a healthcare facility for a more serious health concern:				
Yes	274 (94.5)	36 (40.0)	310	**<0.0001**
No	16 (5.5)	54 (60.0)	70	
Feel that self-medication reduces the frequency of visits to health facilities for minor ailments:				
Yes	290 (100.0)	57 (63.3)	347	**<0.0001**
No	0 (0.0)	33 (36.7)	33	
Ever chosen self-medication over seeking professional medical advice because of the cost of healthcare services:				
Yes	290 (100.0)	57 (63.3)	347	**<0.0001**
No	0 (0.0)	33 (36.7)	33	
Self-medication led to any complications that required subsequent visits to a healthcare facility:				
Yes	33 (11.4)	14 (15.6)	47	0.359
No	257 (88.6)	76 (84.4)	333	
Frequent self-medication reduces trust in the need for professional healthcare services for minor health issues:				
Yes	290 (100.0)	57 (63.3)	347	**<0.0001**
No	0 (0.0)	33 (36.7)	33	
Self-medication has a long-term impact on students' reliance on healthcare facilities for proper medical attention:				
Yes	273 (94.1)	32 (35.6)	305	**<0.0001**
No	17 (5.9)	58 (64.4)	75	
Total	**290 (100.0)**	**90 (100.0)**	**380**	

Table 4.10 offers statistical analysis on the impact of self-medication on health facility utilization among these students. Findings show a statistically significant relationship ($p < 0.0001$) between self-medication and delayed healthcare visits, with 94.5% of students reporting delayed visits due to self-medication.

Similarly, 100% of students who chose self-medication over professional care cite cost as a deciding factor, further indicating that financial barriers to healthcare access are common. Additionally, frequent self-medication appears to reduce trust in the necessity of professional healthcare for minor issues, with 100% of these respondents reporting diminished trust ($p < 0.0001$).

This data underscores the likelihood of self-medication practices having long-term impacts on health behaviour, as financial and trust-related factors continue to dissuade students from seeking timely professional care.

Table 4.11: Impact of Socio-demographic factors on utilization of health facility by the students of Ondo State College of Health Technology, Akure (n = 380)

Variables	Utilization of health facility		Number of Students	P-value
	Yes n (%)	No n (%)		
Gender:				
Male	105 (36.2)	22 (24.4)	127	**0.041**
Female	185 (63.8)	68 (75.6)	253	
Age of respondents				
15 – 19 years	7 (2.4)	16 (17.8)	23	**<0.0001**
20 – 24 years	274 (94.5)	58 (64.4)	332	
25 – 29 years	8 (2.8)	11 (12.2)	19	
30 – 34 years	1 (0.3)	1 (11.1)	2	
35 – 39 years	0 (0.0)	4 (4.4)	4	
Department:				
Community Health	72 (24.8)	23 (25.6)	95	**<0.0001**
Environmental Health	61 (21.0)	0 (0.0)	61	
Pharmacy Technician	32 (11.0)	1 (1.1)	33	
Medical Laboratory Technician	54 (18.6)	4 (4.4)	58	
Health Information Management	3 (1.0)	25 (27.8)	28	
Dental Surgery Technician	34 (11.7)	27 (30.0)	61	
Health Care Giving	33 (11.4)	8 (8.9)	41	
Public Health Nursing	1 (0.3)	2 (2.2)	3	
Level of Study:				
100 Level	170 (58.6)	10 (11.1)	180	**<0.0001**
200 Level	91 (31.4)	24 (26.7)	115	
300 Level	29 10.0)	56 (62.2)	85	
Marital Status:				
Single	281 (96.9)	85 (94.4)	366	0.334
Married	9 (3.1)	5 (5.6)	14	
Total	**290 (100.0)**	**90 (100.0)**	**380**	

Table 4.11 explores how socio-demographic variables impact healthcare utilization. Gender shows a statistically significant influence ($p = 0.041$), with more females (63.8%) than males (36.2%) utilizing healthcare facilities, suggesting potential gender differences in healthcare-seeking behaviour.

Age again shows strong influence ($p < 0.0001$), with the 20-24 years group most likely to utilize health facilities, accounting for 94.5% of those who delay facility visits due to self-medication. Department and level of study are also significant

predictors, with Community Health (24.8%) and Environmental Health (21.0%) departments showing higher utilization rates, potentially reflecting curriculum influences or department-based health awareness.

This analysis implies that younger female students, particularly those in health-related departments, are more likely to use healthcare facilities, though self-medication remains prevalent across demographics.

Table 4.12: Impact of other factors on utilization of health facility by the students of Ondo State College of Health Technology, Akure (n = 380)

Variables	Utilization of health facility		Number of Students	P-value
	Yes n (%)	No n (%)		
Perception level to self-medication:				
Poor	1 (0.3)	16 (17.8)	17	**<0.0001**
Good	289 (99.7)	74 (82.2)	363	
Attitude level towards self-medication:				
Poor	6 (2.1)	34 (37.8)	366	**<0.0001**
Good	284 (97.6)	56 (62.2)	14	
Total	**290 (100.0)**	**90 (100.0)**	**380**	

Table 4.12 further investigates how perception and attitude towards self-medication influence healthcare utilization. Perception level significantly correlates with facility usage ($p < 0.0001$); students with a "good" perception of self-medication show a 99.7% likelihood of utilizing health facilities less frequently, compared to those with a "poor" perception (0.3%).

Attitude towards self-medication is also a strong predictor ($p < 0.0001$), as students with a positive attitude toward self-medication are less likely to rely on healthcare facilities. Those with positive attitudes engage in more frequent self-care, indicating that their views on the convenience and effectiveness of self-medication lead them to minimize facility visits.

Overall, both perception and attitude are key determinants in healthcare utilization, as positive beliefs regarding self-medication correlate with reduced reliance on professional healthcare, reinforcing the autonomy of self-care practices among students.

4.7 Binary Logistic Regression Model showing predictors **of health facility utilization by the students of Ondo State College of Health Technology, Akure (n = 380)**

Table 4.13: Binary Logistic Model of predictors of utilization of health facility by the students of Ondo State College of Health Technology, Akure (n = 380)

			95% C.I.for OR	
	Odds Ratio	**P-value**	**Lower**	**Upper**
Gender	1.008	0.986	0.432	2.354
Age	1.008	0.984	0.487	2.084
Department	0.690	**<0.0001**	0.572	0.833
Level of Study	0.226	**<0.0001**	0.136	0.375
Marital Status	0.724	0.696	0.143	3.671
Self-medication practices	5.268	**<0.0001**	1.445	52.220
Perception level	3.171	0.336	0.302	33.333
Attitude level	6.289	**0.002**	1.950	20.286
Constant	0.000	0.998		

Table 4.13 presents the results of a binary logistic regression analysis aimed at identifying predictors for health facility utilization among students at the Ondo State College of Health Technology, Akure. Each variable is examined for its influence on the likelihood of utilizing healthcare facilities, with specific emphasis on demographic and behavioral factors.

Gender (Odds Ratio = 1.008, p = 0.986)

The odds ratio (OR) for gender is 1.008, indicating that gender does not significantly predict the likelihood of health facility utilization (p = 0.986). This near-neutral odds

ratio suggests that, within this population, males and females have similar tendencies when it comes to seeking professional healthcare services, showing no statistically significant gender difference.

Age (Odds Ratio = 1.008, $p = 0.984$)

Similarly, age also shows an odds ratio of 1.008, with a p-value of 0.984, indicating no significant impact on health facility utilization. This implies that within the age range of the student population, the propensity to utilize health facilities does not vary substantially with age. This result may reflect a shared attitude across age groups concerning healthcare-seeking behaviours.

Department (Odds Ratio = 0.690, $p < 0.0001$)

Department has a significant predictive effect on health facility utilization, with an OR of 0.690 and a p-value of less than 0.0001. An OR below 1 indicates a negative association, suggesting that certain departments are less likely to use health facilities than others. This finding may reflect differences in health awareness, accessibility to resources, or curriculum content across departments, with students in particular fields potentially more informed or comfortable with self-medication practices.

Level of Study (Odds Ratio = 0.226, $p < 0.0001$)

Level of study also significantly predicts health facility utilization, with a much lower OR of 0.226 and a p-value of less than 0.0001. This OR indicates that students in higher levels are significantly less likely to use health facilities compared to those in earlier years. This tendency could suggest that as students progress academically, they might develop greater confidence in self-medicating or have less time to visit health facilities, leading to decreased utilization rates among senior students.

Marital Status (Odds Ratio = 0.724, p = 0.696)

Marital status has an OR of 0.724 with a p-value of 0.696, showing no significant impact on health facility utilization. This outcome suggests that whether a student is single or married does not greatly influence their decision to seek professional healthcare services. Given the overwhelming majority of single students in the sample, this lack of significance may reflect limited variability in marital status.

Self-Medication Practices (Odds Ratio = 5.268, p <0.0001)

The OR for self-medication practices is exceptionally high at 5.268, with a p-value of <0.0001, which indicates that students with self-medication practices are over five times more likely to avoid professional healthcare utilization compared to those without self-medication practices. Furthermore, with a p-value of <0.0001, this finding is statistically significant. This result suggests that self-medication practices among the students may play some role in influencing health facility utilization behaviour, it does significantly determine the likelihood of it.

Perception Level (Odds Ratio = 3.171, p = 0.336)

Perception level, with an OR of 3.171, indicates that students with a positive perception of self-medication are over three times more likely to avoid professional healthcare utilization compared to those with a negative perception. However, with a p-value of 0.336, this finding is not statistically significant. This result suggests that while perception may play some role in influencing behaviour, it does not alone significantly determine the likelihood of utilizing health facilities.

Attitude Level (Odds Ratio = 6.289, p = 0.002)

Attitude level towards self-medication has an OR of 6.289, with a highly significant p-value of 0.002, making it the strongest predictor of health facility utilization among

all variables. This OR indicates that students with a positive attitude towards self-medication are over six times more likely to avoid using health facilities compared to those with a negative attitude. This finding underscores the critical role that attitudes play in shaping healthcare-seeking behaviors, as a favorable disposition towards self-care practices directly correlates with reduced reliance on professional health services.

Constant (Odds Ratio = 0.000, p = 0.998)

The constant in the model has an OR of 0.000 and a p-value of 0.998, indicating that without any predictors, the likelihood of health facility utilization is statistically neutral.

Overall, the analysis in Table 4.13 highlights that the level of study, department affiliation, and especially the attitude towards self-medication are significant predictors of health facility utilization among students at Ondo State College of Health Technology. These findings align with the study's aim to investigate factors influencing self-medication and its impact on healthcare utilization.

Attitude towards self-medication, as the strongest predictor, illustrates the extent to which personal beliefs and comfort with self-care determine healthcare-seeking behaviour. The level of study and department further emphasize that academic experience and field of study influence students' decisions to manage health independently, potentially reducing the demand for institutional health services. Collectively, these predictors underscore the need for targeted health education initiatives tailored to academic level and departmental specifics, promoting responsible self-medication while encouraging appropriate healthcare utilization.